



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : الفت عبد الحكيم قرني حسن

المحافظة : القليوبية الحي / المدينة : شبرا الخيمة اول العنوان : شبرا الخيمة اول

رقم البطاقة : 28410282500441 رقم التليفون : 01143031263 تاريخ تسجيل الإستمارة : 03-10-2026

العمر (تاريخ الميلاد) : 28-10-1984 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....) المقر بما فيه التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي العلاج المساعد لوقاية العظام للمرضي الذين يتلقوا علاج هرموني حمض الزيلودرونك [14157]

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة. تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / / د. / / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: الفت عبد الحكيم قرني حسن National ID: 28410282500441	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Zoledronic acid for bone protection during hormonal therapy
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name : الفت عبدالحكيم فرنى حسن

Sex:

3- Medical number:
Female

83151712

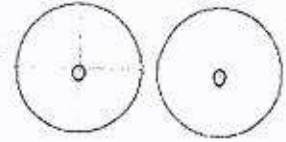
Old Med :

Old Med :

2566-2023

2-ID number: 28410282500441

Patient Age : 39 Y



Family history

Family history

Phone

Phone

01143031263

BMI

BMI

REASON OF MDT

REASON OF MDT

marital status

Marital Status

NRS Score

NRS Score

GROUP

GROUP

Date of MDT

Date of MDT

Co morbidity

Co morbidity

Menopausal
status

Menopausal Status
Pre-menopausal

Initial STAGE

Initial STAGE

07/05/2024

Clinical data

Breast Imaging :

Mets Workup :

BX	ER 7/8	PR 7/8	Her 2 Neu 3	SISH	Ki67 20
BX COMMENT	ER COMMENT	PR COMMENT	Her 2 Neu COMMENT	SISH COMMENT	

T 2	N 2	M 0
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Sonomammography

PATIENT HISTORY

known case of It breast cancer-->underwent upfront BCS+ALND-->PT2 N2 M0 ,inassuit
then received post operative chemotherapy treatment 4 AC last 3/10/2023
BONE SCAN, TAXOTERE/ HERCEPTIN

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

PORT, LHRH and AI for 5 years and consider extended hormonal ttt

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د/ شريف حسنين د/ يحيى اسماعيل	RADIOLOGY	RADIOLOGY د/ شوش عبد القادر
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY د/ انصار محمد القادر
Psychiatric	Psychiatric	Nutrition	Nutrition

Name: الفت عبد الحكيم قرني		
MR. : 4809		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته تاريخ وساعة الدخول : تاريخ وساعة الخروج :		
وبعد الكشف والفحوصات والمتابعة وجد انه The patient has a history of left-sided breast cancer treated with breast-conserving surgery on 31/7/2023. Pathology was T2 N2, grade 6/15, estrogen receptor 7/8, progesterone receptor 7/8, HER2 3+. The patient received four cycles of AC followed by four cycles of TAXOTERE, HERCEPTIN, and HERCEPTIN X17, with the last HERCEPTIN administered in 3/2025. Ongoing endocrine therapy includes ZOLADEX and FEMARA, with ZOLADEX initiated in 5/2024, a planned pause in 8/2025, and a scheduled ZOLADEX dose on 1/2026; the next FEMARA dose is due on 11/4. The patient is currently on ZOLADEX, ZOMETA, and FEMARA. Physical examination was NAD. A repeat CBC is planned for next month, and the total leukocyte count (TLC) is 2.8. The patient continues treatment for breast cancer with adjunctive FEMARA and ZOLADEX administered today. FEMARA is scheduled for a three-month course, and a bone scan performed on 18/4 was reported as free of disease. The patient remains on FEMARA with supportive care. Laboratory investigations from the last follow-up were performed two months ago, and the next follow-up visit is planned after three months. The patient received ZOLADEX, FEMARA, and supportive therapy today, with a bone scan scheduled for 21-6-2026. The patient will be seen in the clinic after three months.		
تحريرا في ,		
	الوقت :	21-09-2026 التاريخ :
	dina makram	الطبيب المعالج :
يعتمد ,		



Patient Name: Olfat Abd Al Hakeem Qorany Hassan	Date : 31/08/2024
Exam : MAMMOGRAPHY BOTH	Age : 39 Year(s)
Medical ID: 2591/2022	Pat. ID: 45776

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

Patient had left CBS in

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- The breasts are) Scattered fibro glandular density (ACR B)
- No right breast suspicious mass lesions, micro calcifications or architectural distortion detected.
- Left breast UOQ focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Complementary ultrasound showed left breast UOQ focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- no right breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex.
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left infra and supra clavicular regions.

Opinion:

- Left breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)
- Normal study of the right breast (BIRADS 1)

Yours truly

Signature Dr. Basma Mohamed

31/08/2024 3:49 PM



Patient Name: Olfat Abd Al Hakeem Qorany Hassan	Date : 30/11/2024
Exam : C.T CHEST	Age : 40 Year(s)
Medical ID: 2566/2023	45776

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS:-

Findings:

↓ Known case of left CBS

I Chest:-

- Distorted parenchyma of the left breast with diffuse skin thickening with no pathological axillary lymph nodes.
- Prominent right axillary lymph nodes being subcentimeteric in short axis and showing preserved fatty hilum, likely reactive for follow up.
- Single pulmonary nodule noted at the anterior segment of right upper lung lobe measures about 5 mm likely inflammatory. No masses, areas of consolidation or cavitary lesions.
- Few prominent mediastinal (prevascular, aorto-pulmonary, carinal and subcarinal) lymph nodes being sub-centimeteric in short axis, likely reactionary.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver mild enlarged with homogenous CT texture. No definite focal lesions (on non contrast base) or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Small right kidney, otherwise left kidney shows normal size, parenchymal thickness & density with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:** a small subcentimeteric lytic focal lesions seen at L2 & L3 vertebral bodies.

Opinion:



Patient Name: Olfat Abd Al Hakeem Qorany Hassan	Date : 30/11/2024
Exam : C.T CHEST	Age : 40 Year(s)
Medical ID: 2566/2023	45776

- Distorted parenchyma with diffuse skin thickening of the left breast for sonomammographic correlation.
- L2 & L3 focal osseous lytic lesions for further evaluation by bone scan.
- Small nephropathic right kidney for lab. correlation.
- No evidence of pulmonary, hepatic metastatic deposits (on non-contrast base).

Marwa Kamal Saber

Dr. Sherif Sameh, M.D.



Patient Name : الأستاذ / الفت عبدالحكيم قرني حسن	Request No. : 13004746
Patient No. : 28410282500441	Request Date : 20/11/2023
Path No. : SR-1432/23	Verification Date : 06/12/2023
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 38 Y	Department : Out Patient
Referring Doc :	Registered by : Ali mostafa

Pathology Report

Clinical data

Breast mass

Nature of specimen

Revision of slides

Gross examination

One block (BH335-A) & One slides were received .

Microscopic examination

* Histological type: Invasive duct carcinoma

* Grade (differentiation +nuclear pleomorphism+mitosis): (3+2+2) Grade II

* DCIS: No

* Angioinvasion: Could not be detected within the limits of biopsy received

IPT :

ER: Moderate nuclear staining in 70% of tumor cells(7/8)

PR: Weak nuclear staining in 30% of tumor cells (4/8)

Her2: Strong complete membrane staining in >10 of tumor cells , score(+3)

KI67: Positive in 30% of tumor cells

Diagnosis

Breast mass ; slide revision :

Invasive duct carcinoma, Grade II .

IPT :

ER: Positive (score 7/8)

PR: Positive (score 4/8)

Her2: Positive score(+3)

ki67: Positive in 30 % of tumor cells

Signature

Dr. Marwa Mahmoud

Verified by :

Dr. Heba Hasan

Printing Date : 10/12/2023 09:33:17

Page 1 of 2

Printing By : marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name : الأستاذة / الفت عبدالحكيم قرني حسن
Patient No. : 28410282500441
Path No. : 1875/23
Sex : Female
Age : 38 Y
Referring Doc. :

Request No. : 13002557
Request Date : 12/06/2023
Verification Date : 11/07/2023
Financial Category : أورام دار السلام - نفقة الدولة
Department : Out Patient
IHC No. :
Registered by : marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass lesion

Nature of Specimen: Tru-cut biopsy, collected at 11/6/2023. 11:58 4m.

Gross Examination:

- Fragmented greyish white tissue cores up to 1.3 cm, Totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma
- * Grade (differentiation+ nuclear pleomorphism + mitosis): Grade II (3+2+1).
- * Angioinvasion: Couldn't be detected in sections examined
- * Ductal carcinoma in situ: Present
- * IPT (Positive control showed appropriate reactivity):
- ER: Moderate nuclear reaction in 70% of tumor cells (Allred score 7/8).
- PR: Moderate nuclear reaction in 70% of tumor cells (Allred score 7/8).
- Her2: Strong complete membrane reaction in >10% tumor cells (score 3+).
- Ki67: 20% nuclear reaction of tumor cells.

Diagnosis:

Left breast mass lesion, Tru cut biopsy:

Invasive duct carcinoma, Grade II, with DCIS.

IPT:

- ER: Positive (score 7/8).
- PR: Positive (score 7/8).
- Her2: Positive (score 3+).
- Ki67: Positive in 20% of tumor cells.

Verified by :

Dr. Hebatallah Mostafa

Printing Date : 12/07/2023.10:27:29

Page 1 of 2

Printed By : marwa mohamed youss

Request No.	13002557
Request Date	12/06/2023
Verification Date	11/07/2023
Financial Category	اورام ناز السلام - نفقة الدولة
Department	Out Patient

IMMUNOHISTOCHEMISTRY REPORT

Signature

إدارة رقم (4) - الدور الأول - مبنى رقم (987) - فورلايك - لبنان - مجلس الجامعة - اللاذقية - سورية
Hannu Nlp. (4) - First Floor, Building No. 987, Nita Cusackin - After 11 Quetema, Coten
Lynne

Assiut University
South Egypt Cancer Institute
Radiology department



جامعة أسيوط
معهد جنوب مصر للأورام
قسم الأشعة التشخيصية

Name : Olfat Abd EL Hakeem Karny 39

Examination: Contrast Enhanced MSCT examination of the Chest & Abdomen.

Date: 27/8/2023

REPORT

• Known case of breast cancer underwent BCS : No available previous images for comparison :

• As regard the chest:

- Both lungs show normal appearance of their parenchyma with preserved pulmonary architecture.
- No definite metastatic pulmonary nodules or masses.
- Normal CT appearance of the mediastinal vessels.
- Noted multiple enlarged mediastinal LNs , the largest SAD measures +/- 9mm
- Patent tracheo bronchial tree
- No pleural effusion.
- Noted multiple left supraclavicular LNs , the largest SAD measures +/- 5mm.
- Noted increased left breast skin thickness measures +/- 15 mm...for Sonomammographic correlation.

• As regard the abdomen and pelvis:

- Moderately enlarged Liver with smooth surface, homogenous density and post contrast enhancement. No definite hepatic focal lesions. No intra hepatic biliary channels or venous channels dilatation.
- Normal caliber and opacification of the PV with no definite intra-luminal thrombosis.
- Mildly enlarged spleen with normal shape and homogenous density.
- GB: Average capacity, normal wall thickness with no stones inside, no masses. Normal C.B.D. diameter.
- Average sized pancreas with normal shape and density.
- Right kidney: Normal site, small sized with good cortical thickness, smooth surface and clear perinephric fat. No renal stones or masses. No backpressure changes.
- Left kidney: Normal site, average sized with good cortical thickness, smooth surface and clear perinephric fat. No renal stones or masses. No backpressure changes.
- U.B: Average capacity, uniform low density fluid content with normal wall thickness and no stones or mass lesions.
- Noted multiple para aortic, aortocaval and bilateral iliac LNs, the largest SAD measures +/- 7mm.
- Minimal pelvic ascites
- Noted right simple adnexal cyst measures +/- 2x1.5 cm.



Patient Name: Olfat Abd Al Hakeem Qorany Hassan	Date : 02/09/2024
Exam : MRI BRAIN WITH CONTRAST	Age : 40 Year(s)
Medical ID: 2566/2023	Pat. ID: 45776

MRI STUDY OF THE BRAIN

Technique of Examination:

- Axial T1, T2 & FLAIR WIs
- Sagittal T1 WIs
- Post contrast series

MR Findings:

- No definite enhancing intra cranial masses.
- No cerebral parenchymal areas of abnormal signal.
- Normal grey / white matter differentiation.
- Normal size and configuration of the ventricular system.
- No shift of midline structures.
- No areas of fresh blood or blood degradation product signal.
- Normal appearance of the brain stem and cerebellum.
- No extra axial collections.

Opinion:

No definite enhancing intra cranial masses.

Dr, Hala Sayed, MSc

Dr. Sherif Sameh, M.D.

Patient name: ألفت عبد الحكيم قرني
Clinical information: Breast cancer.
Referred from: د. محمود الشعيبي
Receiving date: 24/07/2023

Age: 35 years

Lab No.: BH335

Report date: 31/07/2023

Histopathology Report

Nature of specimen:

Wide local excision with safety margins and axillary lymph nodes. Surgical margins were labeled by the surgeon by different sutures (single silk suture was labelling the upper outer margin and double silk sutures were labelling the medial margin). The specimen was received in two containers and labeled as following:

1. Wide local excision with safety margins.
2. Extended lateral margin + axillary lymph nodes.

Gross Description:

1. Received in saline was a single soft yellow fibrofatty tissue piece measured 12x10x5 cm covered by ellipse of skin measured 6x3 cm. Serial cut sections showed a firm white mass measured 2x2x1 cm. Another well defined mass was seen measured 3x2x2 cm and showed cystic degeneration. Surgical margins were labeled by different sutures (single silk suture was seen labelling the upper margin and double silk sutures were seen labelling the medial margin). Representative sections were submitted.
2. Multiple fibrofatty tissue pieces measured 7x5x4 cm and contained 15 lymph nodes measured 0.5-3 cm with attached extended lateral margin.

Microscopic Description:

Microscopic examination of sections from both breast masses revealed invasive tumor tissue displaying pleomorphic epithelioid cells existed in nests, trabeculae, acini and separate forms within sclerotic stroma with mild lymphocytic infiltrate. The neoplastic cells showed nuclear enlargement, high N/C ratio, coarse clumped chromatin and few mitotic figures. The neoplastic cells displayed tubular pattern score 2, nuclear atypia score 3, and mitotic activity score 1 (modified Bloom-Richardson system). Foci of ductal carcinoma in situ, grade II and III, solid and comedo types were seen about 30% of tumor tissue. Foci of perineural invasion and lymphovascular tumor emboli were seen. Sections from 15 LNs showed metastatic tumor deposits at 6 of them. Surgical margins (upper, lower, medial, extended lateral and deep surgical margins) were free from neoplasia.

Diagnosis:

Breast conservative surgery with safety margins and axillary lymph nodes dissection:

- Infiltrating duct carcinoma (NOS), grade 2 (modified Bloom-Richardson system / Nottingham histologic grade, 2006), multifocal, the largest mass measured 3 cm in greatest dimension.
- Ductal carcinoma in situ, grade II and III solid and comedo types (about 30% of tumor).
- Perineural invasion and lymphovascular tumor emboli were seen.
- Tumor infiltrating lymphocytes (TILs) score was low (about 10% of tumor).
- Surgical margins (upper, lower, medial, extended lateral and deep surgical margins) were free.
- 6 out of 15 LNs showed tumor deposit (6/15).
- pT2, N2a (AJCC, 8th edition, 2017).

Signature:
Dr. Mahmoud Farouk Sherif, MD, FIAC

Signature

Dr. Mahmoud Farouk Sherif, MD, FIAC

٧١٢١



Patient Name	الأستاذة / الفت عبدالحكيم قرني حسن	Request No.	13002557
Patient No.	28410282500441	Request Date	12/06/2023
Path No.	1875/23	Verification Date	11/07/2023
Sex	Female	Financial Category	اورام دار السلام - نفقة الدولة
Age	38 Y	Department	Out Patient
Referreing Doc.			
		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Signature
Prof. Dr. Gamal Abdelaty
Dr. Rana Fathy
Dr. Walaa Ghonain



Verified by :
Dr.Hebatallah Mostafa

Printing Date : 12/07/2023 10:27:29

Page 2 of 2

Printed By : .marwa mohamed youss

Nuclear Medicine Unit

Patient Name: Olfat Abdel Hakeem Karni

Date: 2/2/2025

Referring physician: Prof.

 ^{99m}Tc -MDP BONE SCANDesired information: skeletal surveying to detect multiple lesions.Technique: Anterior and posterior whole-body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc -MDP:Report:

- No scintigraphic evidence of bony metastases .
- Heterogenous abnormal radiotracer uptake is seen at the spine , more determined at , mid dorsal , lumbar vertebrae , likely of spondylodegenerative changes ,
 - Osteoarthritic changes are seen at both knees , both shoulders , .
- Both kidneys are normally seen.

Conclusion:

No definite hot spot that could account for osseous metastatic lesions

Thanks for the referral

Nuclear Medicine Unit

DR. Sherin Mahdy, MD

DR/ Mohamed Emam